

1. A 52-year-old woman comes to the office for a health maintenance examination. She has no history of serious illness and takes no medications. Menopause occurred 1 year ago. She does not exercise. Her diet consists mainly of meat and potatoes with very few vegetables. She drinks one beer daily and four to five large cups of coffee daily. She had smoked one pack of cigarettes daily for 20 years, but quit 10 years ago. She works as a mechanic. She has no family history of cancer, but wishes to know what she can do to reduce her risk for breast cancer. The patient appears well. She is 168 cm (5 ft 6 in) tall and weighs 95 kg (210 lb); BMI is 34 kg/m². Vital signs are within normal limits. Physical examination shows no abnormalities. Which of the following is most likely to reduce this patient's risk for breast cancer?
- ☐ A) Abstinence from caffeine
 - ☐ B) Alcohol cessation
 - ☐ C) Exercising and reducing her caloric intake
 - ☐ D) Increasing her intake of soy
 - ☐ E) Raloxifene therapy



Next



Lab Values



Calculator



Review



Help



Pause

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Family Medicine Self AssessmentTime Remaining:
1 hr 13 min 38 sec

2. A state-supported academic health and insurance system is contracted across a large state in the western United States to cover the aspects of care for patients of all ages and income levels, including the Medicaid and Medicare populations. The system's goal is to provide the highest quality of care in the most efficient manner to all patients equally and to support the productivity of the population in that state. The organization already has many skilled providers within every specialty, but they also wish to prioritize key areas for proactive improvement. Which of the following strategies is most likely to be effective in achieving this goal?
- ☐ A) Eliminate prior authorization requirements and expand drug formularies to include more prescription options
 - ☐ B) Establish a mobile clinic to bring specialty services directly to underserved areas of the state
 - ☐ C) Expand insurance access for specialty, advanced imaging, and surgical services
 - ☐ D) Expand telemedicine services
 - ☐ E) Increase relative investment in primary care services

3. A 6-year-old girl is brought to the office by her mother for a well-child examination. The mother is concerned about nighttime bedwetting, reporting that although the patient has been toilet-trained for the past 2 years during the daytime, she has "never had a dry night." The patient has three siblings who the mother states were all dry through the night by age 5 years. The patient is otherwise healthy and vaccinations are up-to-date. Vital signs are within normal limits and physical examination shows no abnormalities. Fasting fingerstick blood glucose concentration is 82 mg/dL, and urinalysis is within the reference ranges. Which of the following is the most appropriate next step in management?

- ☐ A) Desmopressin therapy
- ☐ B) Fluid restriction after 2 PM
- ☐ C) Renal ultrasonography
- ☐ D) Urine culture
- ☐ E) Reassurance only



Previous



Next



Lab Values



Calculator



Review



Help



Pause

4. A 44-year-old man comes to the office because of a 5-year-history of a lump on his upper back. Four months ago, the lump became tender for 1 week when he had been exercising at the gym more often, but the tenderness resolved spontaneously. He says the lump has not changed in size and he has not noticed any skin changes. The patient is worried that the lump could be cancerous. He has no history of serious illness and takes no medications. He is 183 cm (6 ft) tall and weighs 82 kg (180 lb); BMI is 24 kg/m². Vital signs are within normal limits. Physical examination shows a 4-cm nontender, moderately firm, mobile mass between the right scapula and the spine. No other masses are palpated. Ultrasonography of the lump shows a well-defined mass with a homogeneous echotexture. Which of the following is the most appropriate next step in management?

- ☐ A) Excisional biopsy
- ☐ B) Fine-needle biopsy
- ☐ C) MRI of the thoracic spine
- ☐ D) Repeat ultrasonography in 3 months
- ☐ E) Continued observation

5. A previously healthy 19-year-old man comes to the physician because of a 1-week history of sore throat, difficulty swallowing, and fever. He has a muffled voice. His temperature is 39.4°C (103°F). Which of the following would warrant the need for surgical intervention?

- ☐ A) Anterior cervical lymphadenopathy
- ☐ B) Lateral deviation of the uvula
- ☐ C) Petechiae over the soft palate
- ☐ D) Pharyngeal exudates
- ☐ E) Tenderness of the trachea

6. A 37-year-old man comes to the physician because of a 6-week history of epigastric pain. The pain is most severe 1 hour after eating and is relieved by over-the-counter antacids. He has had no fever, nausea, or weight loss. He has hypertension treated with hydrochlorothiazide. He does not smoke and drinks one glass of wine with dinner daily. His pulse is 76/min, and blood pressure is 136/88 mm Hg. There is mild epigastric tenderness without rebound or guarding. The stool is brown, and test for occult blood is negative. His hemoglobin concentration is 13.8 g/dL. Liver function tests show no abnormalities. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Esophageal pH monitoring
- ☐ B) Measurement of serum amylase activity
- ☐ C) Serum *Helicobacter pylori* antibody assay
- ☐ D) Abdominal ultrasonography
- ☐ E) Abdominal CT scan

7. A previously healthy 47-year-old woman comes to the physician because of a 4-week history of palpitations and feeling hot, sweaty, and anxious. During the past 2 months, she has had a 3.2-kg (7-lb) weight loss despite a normal appetite. She has not had chest pain, shortness of breath, light-headedness, dizziness, or nausea. Her last menstrual period was 3 weeks ago. Menses previously occurred at regular 30-day intervals, but she notes that during the past year she "might have missed a few periods." She does not smoke cigarettes. She drinks one glass of wine monthly. She is 165 cm (5 ft 5 in) tall and weighs 61 kg (135 lb); BMI is 23 kg/m². Her pulse is 112/min and regular, respirations are 16/min, and blood pressure is 138/88 mm Hg. Examination shows no other abnormalities. Her serum thyroid-stimulating hormone concentration is 0.1 μ U/mL. Results of additional laboratory studies are pending. Which of the following is the most appropriate next step in management?
- ☐ A) Admission to the hospital
 - ☐ B) Endometrial biopsy
 - ☐ C) Fine-needle biopsy of the thyroid gland
 - ☐ D) ¹³¹I therapy
 - ☐ E) Thyroid ultrasonography

8. A 25-year-old woman comes to the office because of a 6-month history of difficulty sleeping and "feeling worried and edgy most days." She has been a patient for the past 10 years. She has no history of serious illness. Her only medication is an oral contraceptive. She works as an emergency room technician but is applying to nursing school next year. Vital signs are within normal limits. Physical examination shows no abnormalities. On mental status examination, she has an anxious mood and jittery affect. Which of the following is the most appropriate next step in management?

- ☐ A) Alprazolam therapy
- ☐ B) Diphenhydramine therapy
- ☐ C) Office-based counseling
- ☐ D) Pregabalin therapy
- ☐ E) Valerian root supplementation

9. A 14-year-old boy is brought to the physician because his parents are concerned that he is shorter than his classmates. He is at the 10th percentile for height and 25th percentile for weight. Both parents are at the 75th percentile for height. His diet consists of cereal, milk, bread, and fried food. The patient's temperature is 37°C (98.6°F), pulse is 75/min, respirations are 12/min, and blood pressure is 110/65 mm Hg. Examination shows sparse facial hair and sparse, straight, slightly pigmented hair at the base of a slightly enlarged penis. The skin is warm and moist. The testes are approximately 3 cm long and are descended into the scrotum bilaterally. X-rays show a bone age of 12 years (± 4 months). Which of the following is the most likely cause of these findings?

- ☐ A) Constitutional growth delay
- ☐ B) Familial short stature
- ☐ C) Hypothyroidism
- ☐ D) Nutritional deficiency
- ☐ E) Normal development

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Family Medicine Self AssessmentTime Remaining:
58 min 3 sec

The following vignette applies to the next **2** items. The items in the set must be answered in sequential order. Once you click **Proceed to Next Item**, you will not be able to add or change an answer.

A 70-year-old man comes to the physician because of a 1-week history of progressive shortness of breath and swelling of his legs. The shortness of breath now occurs after he walks for only a few minutes, limiting his ability to climb stairs. He uses two pillows to sleep comfortably. During the past week, he has had a 4.5-kg (10-lb) weight gain despite a normal appetite. He has a 10-year history of metabolic syndrome and a 6-year history of chronic obstructive pulmonary disease. Seven years ago, he sustained a myocardial infarction and subsequently underwent uncomplicated coronary artery bypass grafting of four arteries. His medications are furosemide, clopidogrel, metoprolol, lisinopril, and aspirin. Three days ago, he doubled his dosage of furosemide without the physician's knowledge. On questioning, he says he has not had increased urine output. He has smoked one pack of cigarettes daily for 50 years. His pulse is 110/min, respirations are 22/min, and blood pressure is 160/95 mm Hg. Pulse oximetry on room air shows an oxygen saturation of 86%. Examination shows jugular venous distention. Crackles are heard halfway up both lungs. An S₃ is heard; no murmurs are heard. There is 3+ pitting edema of the lower extremities.

10. Item 1 of 2

Which of the following is the most likely cause of this patient's current symptoms?

- ☐ A) Chronic obstructive pulmonary disease
- ☐ B) Community-acquired pneumonia
- ☐ C) Ischemic cardiomyopathy
- ☐ D) Metabolic syndrome
- ☐ E) Pulmonary embolus

[Proceed to Next Item](#)

You must select "Proceed to Next Item" to view the next item.

Exam Section : Item 10-11 of 50

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Family Medicine Self AssessmentTime Remaining:
55 min 10 sec

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10. Item 1 of 2

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11. Item 2 of 2

The patient is admitted to the hospital. Echocardiography shows a left ventricular ejection fraction of 20%, global hypokinesis, and no evidence of myocardial infarction. In addition to continued furosemide, which of the following is the most appropriate pharmacotherapy?

- ☐ A) Aldosterone antagonist
- ☐ B) Angiotensin II receptor blocking agent
- ☐ C) Calcium-channel blocking agent
- ☐ D) Cardiac glycoside
- ☐ E) Long-acting nitrate

12. A 72-year-old man comes to the physician because of a 2-day history of moderate back pain, fever, chills, and nausea. During this time, he also has had a burning sensation with urination and urinary urgency. He has a 3-year history of benign prostatic hyperplasia characterized by incomplete voiding and intermittent urges to void during the night. His medications are tamsulosin and aspirin. He appears ill. His temperature is 39.1°C (102.3°F). Percussion of the back elicits moderate discomfort. Abdominal examination shows diffuse tenderness to palpation but no rebound or guarding. Urine dipstick testing shows 2+ blood, positive nitrites, and positive leukocyte esterase. Results of urine culture are pending. Which of the following is the most likely causal organism?

- ☐ A) *Citrobacter freundii*
- ☐ B) *Escherichia coli*
- ☐ C) *Klebsiella pneumoniae*
- ☐ D) *Proteus mirabilis*
- ☐ E) *Staphylococcus saprophyticus*

13. A 10-year-old boy is brought to the office for a well-child examination. He and his family emigrated from Bolivia to the USA 7 years ago. He has no history of serious illness and receives no medications. Immunizations are up-to-date. He lives in a house built in 1965. Development is appropriate for age. He is at the 68th percentile for height, 96th percentile for weight, and greater than 95th percentile for BMI. Vital signs are within normal limits. Examination shows no other abnormalities. Which of the following is the most appropriate screening at this time?
- ☐ A) Measurement of blood lead concentration
 - ☐ B) Measurement of fasting serum glucose concentration
 - ☐ C) Measurement of serum hepatitis B surface antibodies
 - ☐ D) PPD skin test
 - ☐ E) Urinalysis

14. A 9-month-old girl is brought to the physician for a well-child examination. Developmental milestones are appropriate for age. She is at the 50th percentile for length and 60th percentile for weight. Her temperature is 37°C (98.6°F), pulse is 137/min, respirations are 32/min, and blood pressure is 86/42 mm Hg. Examination shows no abnormalities other than mild conjunctival pallor. Her hematocrit is 29%, and mean corpuscular volume is 65 μm^3 . Which of the following questions would be most helpful to ask the patient's parents to determine the cause of this child's anemia?

- ☐ A) "Did she have jaundice at birth?"
- ☐ B) "Does she bruise easily?"
- ☐ C) "Has anyone in the family had their spleen removed?"
- ☐ D) "Has she passed any blood with her urine?"
- ☐ E) "What does she eat?"

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48 min 38 sec

15. A 4-year-old boy is brought to the office because of a 1-day history of moderate left hip pain. He walks with a limp. Ten days ago, he had a mild cough and runny nose that resolved spontaneously. He has not sustained any trauma. He has no other history of serious illness and receives no medications. He appears well. Vital signs are within normal limits. On examination, the left hip is in normal position. Palpation of the left hip joint produces no tenderness. Active and passive range of motion is limited by pain. Examination of all other joints shows no abnormalities. Results of laboratory studies are pending. Ultrasonography of the left hip joint shows a small amount of fluid. Which of the following is the most appropriate next step in management?

- ☐ A) Ceftriaxone therapy
- ☐ B) Ibuprofen therapy
- ☐ C) MRI of the left hip
- ☐ D) Vancomycin therapy
- ☐ E) X-ray of the left hip

16. A 16-year-old boy comes for an examination prior to participation in school sports. A painless mass is palpated in the scrotum; the mass decreases while supine and has a bag-of-worms sensation. The boy states that he did not notice it previously. Which of the following is the most likely diagnosis?

- ☐ A) Epididymitis
- ☐ B) Hydrocele
- ☐ C) Inguinal hernia
- ☐ D) Leydig cell tumor
- ☐ E) Teratoma of the testis
- ☐ F) Torsion of the testis
- ☐ G) Varicocele

17. A 57-year-old woman comes to the office because she is concerned about her risk for osteoporosis. Her 65-year-old sister recently sustained a hip fracture. One year ago, the patient sustained a fracture of her right wrist in a fall. She has no other history of serious illness. Menopause occurred 10 years ago. Her only medication is a daily multivitamin. She has smoked one pack of cigarettes daily for 30 years. She has a sedentary lifestyle. She is 152 cm (5 ft) tall and weighs 50 kg (110 lb); BMI is 22 kg/m². Her pulse is 64/min, and blood pressure is 120/60 mm Hg. Examination shows no abnormalities. The physician advises her to stop smoking, begin calcium supplementation, and begin a weight-bearing exercise program. Which of the following is the most appropriate next step in management?
- ☐ A) Bisphosphonate therapy
 - ☐ B) DEXA scan at the age of 65 years
 - ☐ C) DEXA scan now
 - ☐ D) Estrogen and progestin replacement therapy
 - ☐ E) Follow-up examination in 1 year

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44 min 47 sec

18. A 15-year-old girl is brought to the physician because of left ear pain for 2 days. She competes in long-distance open-water swimming events and trains in the ocean several hours daily. Vital signs are within normal limits. Examination of the left ear shows erythema and edema of the auditory canal and a small amount of white discharge. Manipulation of the left pinna produces pain; the left tympanic membrane appears normal. After treating this patient for her acute condition, recommendation of which of the following is the most appropriate next step to prevent recurrence of this condition?

- ☐ A) Training in chlorine-treated pools only
- ☐ B) Daily use of topical benzocaine in the ear canal
- ☐ C) Daily use of alcohol-acetic acid ear drops
- ☐ D) Daily use of carbamide peroxide ear drops
- ☐ E) Daily use of hydrocortisone ear drops

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43 min 6 sec

19. A 31-year-old woman comes to the office because of a 2-month history of progressive fatigue and lack of energy. During this time, she also has had a 2.3-kg (5-lb) weight loss despite no changes in appetite or exercise. She has a 2-year history of frequent diarrhea. She has not had any abdominal pain but does have flatulence, which is worse when she runs. She is an avid runner and is preparing for her sixth marathon. Her medications are an oral contraceptive and over-the-counter loperamide as needed. She traveled to Peru 8 months ago. She is 168 cm (5 ft 6 in) tall and weighs 66 kg (145 lb); BMI is 23 kg/m². Her temperature is 37.2°C (99.0°F), pulse is 60/min, respirations are 12/min, and blood pressure is 110/64 mm Hg. Examination shows no abnormalities. Laboratory studies are shown:

Hemoglobin	11 g/dL
Hematocrit	33%
Mean corpuscular volume	72 µm ³
Leukocyte count	6700/mm ³
Serum ferritin	30 ng/mL

Results of a basic metabolic profile are within the reference ranges. Which of the following is the most likely mechanism of this patient's diarrhea?

- ☐ A) Amyloid infiltration of the bowel
- ☐ B) Bacterial invasion of the bowel wall
- ☐ C) Enzyme deficiency
- ☐ D) Intestinal hypermotility
- ☐ E) Villous atrophy
- ☐ F) Vitamin deficiency

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41 min 55 sec

20. An 86-year-old man is admitted to the hospital because of a 4-day history of cough, diagnosed in the emergency department as pneumonia. The patient's medical record includes a signed and notarized living will dated 2 years ago that indicates his desire for no cardiopulmonary resuscitation or ventilation in the event of incapacitation, and administration of comfort measures only. Upon admission, the patient requests full resuscitation if his status should decline. The patient is oriented to person, place, and time. Initially, only oxygen via nasal cannula and antibiotics are administered, but overnight the patient's status worsens. In the morning, pulse oximetry on 40% oxygen administered via face mask shows an oxygen saturation of 85%. Which of the following is the most appropriate next step in management?

- ☐ A) Adhere to the living will and provide morphine for comfort
- ☐ B) Ask the patient why his wishes changed
- ☐ C) Contact the hospital ethics committee to provide guidance
- ☐ D) Contact the patient's family to clarify his preferences
- ☐ E) Provide ventilator support

21. A 24-year-old man comes to the office because of a 1-week history of fever, sore throat, and diarrhea. The patient says that he was evaluated in an urgent care clinic 3 days ago; results of a rapid streptococcal test and heterophile antibody test for infectious mononucleosis obtained at that time were negative. Symptomatic treatment for viral respiratory infections was recommended, but the patient did not obtain any from his pharmacy. He has no history of serious illness and takes no medications. The patient reports that he has had oral-receptive and anal-receptive sex with men and that his most recent sexual encounter was 2 weeks ago. He does not use condoms. Today, temperature is 38.5°C (101.3°F), pulse is 96/min, respirations are 16/min, and blood pressure is 124/78 mm Hg. Physical examination shows three oral ulcerations and anterior cervical lymph nodes that are slightly tender to palpation. The remainder of the physical examination shows no abnormalities. Which of the following studies is most likely to establish the diagnosis?
- ☐ A) Epstein-Barr virus DNA detection
 - ☐ B) Epstein-Barr virus specific antibodies
 - ☐ C) HIV antibody testing
 - ☐ D) HIV viral load
 - ☐ E) Throat culture for *Neisseria gonorrhoeae*
 - ☐ F) Throat culture for streptococcal pharyngitis

22. During a routine health maintenance examination, a 15-year-old girl says that she has used marijuana with friends on four occasions. She is healthy and doing well in school. She requests that her parents not be told about the marijuana use. In addition to counseling her about the adverse effects of marijuana use, which of the following is the most appropriate intervention at this time?
- ☐ A) Referral to a psychologist for treatment of substance use disorder
 - ☐ B) Scheduling a family conference
 - ☐ C) Scheduling a follow-up examination with the patient
 - ☐ D) Screening for human papillomavirus infection
 - ☐ E) Urine toxicology screening

23. A 47-year-old woman comes to the physician because of a 1-week history of progressive pain and swelling of the left knee. She has pain with walking and at rest. There is no history of trauma. She has hypertension, type 1 diabetes mellitus, and asthma. Her current medications are insulin, a β -adrenergic blocking agent, and a daily inhaled corticosteroid. She is 158 cm (5 ft 2 in) tall and weighs 136 kg (300 lb); BMI is 55 kg/m². Her temperature is 37.8°C (100.1°F); other vital signs are within normal limits. The knee is tender, swollen, warm, and erythematous. There is severe pain with passive and active range of motion. Plain x-rays of the knee show periarticular osteopenia; the joint space is narrowed medially. Which of the following is the most appropriate next step in management?

- ☐ A) Measurement of serum uric acid concentration
- ☐ B) MRI of the knee
- ☐ C) Aspiration of the knee joint
- ☐ D) Intra-articular injection of corticosteroids
- ☐ E) Naproxen therapy

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37 min 9 sec

24. A 55-year-old man comes to the physician with his wife because of a 6-month history of daytime drowsiness. His wife reports that he has had increased snoring, which is especially loud when he sleeps on his back, but he does not stop breathing for prolonged periods. The patient has a 15-year history of hypertension treated with hydrochlorothiazide. He is 175 cm (5 ft 9 in) tall and weighs 82 kg (180 lb); BMI is 27 kg/m². Vital signs are within normal limits. Examination shows an increased intensity of the pulmonic component of S₂. Which of the following is the most appropriate next step in management?

- ☐ A) ECG
- ☐ B) Echocardiography
- ☐ C) Methylphenidate therapy
- ☐ D) Modafinil therapy
- ☐ E) Polysomnography
- ☐ F) Sleep positioning therapy

25. A 65-year-old woman comes to the office as a new patient to refill her prescriptions after relocating to the area. She reports feeling well. She has a history of hypothyroidism, bipolar disorder, and schizophrenia, which are well-controlled with lithium carbonate, nortriptyline, olanzapine, and levothyroxine. Vital signs are within normal limits. Physical examination discloses a flat affect; no additional abnormalities are noted. Results of laboratory studies are shown:

Hemoglobin A _{1c}	5.0%
Serum	
Na ⁺	139 mEq/L
K ⁺	5.0 mEq/L
Ca ²⁺	11.9 mg/dL
Urea nitrogen	26 mg/dL
Creatinine	1.6 mg/dL
Albumin	4.6 g/dL
LDL-cholesterol	100 mg/dL
Thyroid-stimulating hormone (TSH)	1.2 µU/mL
AST	25 U/L
ALT	22 U/L
25-hydroxyvitamin D	42 ng/mL (N>20)

Results of HIV antibody and hepatitis C testing are negative. After reviewing the patient's results, a follow-up examination is scheduled for 3 days from today. In addition to measuring serum lithium concentration, which of the following tests is most appropriate to perform at this time?

- ☐ A) Bone density test
- ☐ B) Renal ultrasonography
- ☐ C) Serum parathyroid hormone concentration
- ☐ D) Urinary calcium concentration
- ☐ E) X-ray of the spine

26. An asymptomatic 42-year-old man with a 1-month history of essential hypertension comes for a follow-up examination. He has been moderately obese for 20 years and follows a high-calorie, high-fat diet. He drinks four to five cups of coffee daily. He has smoked one pack of cigarettes daily for 20 years. He drinks one beer at dinner almost daily. He has a sedentary lifestyle. He is 175 cm (5 ft 9 in) tall and weighs 100 kg (220 lb); BMI is 33 kg/m². His pulse is 78/min, and blood pressure is 148/94 mm Hg. Examination shows no other abnormalities. Which of the following is the most effective method to decrease his blood pressure?

- ☐ A) Abstinence from alcohol
- ☐ B) Abstinence from coffee
- ☐ C) Smoking cessation
- ☐ D) Weight loss of 9 kg (20 lb)
- ☐ E) Biofeedback
- ☐ F) Vitamin E supplementation

27. An 88-year-old woman comes to the office for a routine health maintenance examination. She has osteoporosis treated with alendronate. She also takes a vitamin D supplement. The patient underwent a DEXA scan within the past year; she demonstrated a stable bone density with a T-score of -2.3. She is still active in her community, but she lives alone. BMI is 21 kg/m². She has never smoked cigarettes and does not drink alcoholic beverages. Vital signs are within normal limits. Physical examination discloses no abnormalities. In addition to refilling her prescription for alendronate, which of the following is the most appropriate next step in management?
- ☐ A) Obtain depression screening
 - ☐ B) Obtain serum thyroid-stimulating hormone concentration
 - ☐ C) Order mammography
 - ☐ D) Order mental status examination
 - ☐ E) Schedule a follow-up DEXA scan now

28. A 77-year-old woman comes to the physician because of a 1-week history of decreased appetite, nausea, and occasional vomiting. She also reports fullness and discomfort in her abdomen and abdominal cramps that have awakened her on several occasions. Her last bowel movement was 5 days ago and was characterized by loose stool. Thirty years ago, she underwent hysterectomy for leiomyomata uteri. She has a 10-year history of intermittent urinary incontinence. Her only medication is oxybutynin. Colonoscopy 3 years ago showed no abnormalities. She appears frail. She is 157 cm (5 ft 2 in) tall and weighs 57 kg (125 lb); BMI is 23 kg/m². Her temperature is 37°C (98.6°F), pulse is 80/min and regular, respirations are 14/min, and blood pressure is 130/85 mm Hg. Examination shows warm, dry skin. The lungs are clear to auscultation and percussion. Cardiac examination shows no murmurs. The abdomen is soft and diffusely tender; there is no rebound or organomegaly. Rectal examination shows firm stool in the rectal vault. Which of the following is the most likely cause of this patient's current symptoms?

- ☐ A) Adhesions
- ☐ B) Colon cancer
- ☐ C) Diverticulitis
- ☐ D) Fecal impaction
- ☐ E) Ischemic bowel
- ☐ F) Volvulus

29. A 22-year-old man comes to the office for a follow-up visit. He received the diagnosis of *Chlamydia trachomatis* infection 3 months ago. He was tested after his partner was found to be infected. The patient has never been symptomatic. He and his partner were treated with azithromycin. He has had five lifetime female sexual partners; he has used condoms inconsistently. He does not use illicit drugs. Examination shows no genital lesions. In addition to *Neisseria gonorrhoeae* and HIV testing, this patient should receive testing for which of the following sexually transmitted diseases?

- ☐ A) *Chlamydia trachomatis*
- ☐ B) Herpes simplex virus, type 1
- ☐ C) Herpes simplex virus, type 2
- ☐ D) Human papillomavirus
- ☐ E) No further testing is indicated

30. A 70-year-old man with type 2 diabetes mellitus and hypertension comes to the physician for a routine examination. Six months ago following a myocardial infarction, he underwent coronary artery bypass grafting that involved harvesting of the saphenous vein. He says he feels well. His medications are metformin, metoprolol, and lisinopril. Before his operation, he walked 3 miles daily; he has not resumed walking because he says he lacks the desire. On further questioning, he says he has not resumed any social activities because he feels tired on most days and does not want to leave his house. His blood pressure is 140/80 mm Hg; other vital signs are within normal limits. On examination, there is lower extremity edema bilaterally, which is worse on the left. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Administration of the Geriatric Depression Scale
- ☐ B) Determination of ankle brachial indices
- ☐ C) Determination of erythrocyte sedimentation rate
- ☐ D) Doppler ultrasonography of the lower extremities
- ☐ E) Echocardiography

31. Of 65 men with coronary artery disease, 38 had an abnormal exercise stress test. Which of the following characteristics of screening tests does this demonstrate?

- ☐ A) False-negative rate
- ☐ B) Likelihood ratio
- ☐ C) Predictive value of a positive test
- ☐ D) Sensitivity
- ☐ E) Specificity

32. A 16-year-old girl comes to the physician for a routine health maintenance examination. She says that she began following a vegan diet (no dairy or animal products) 2 months ago. She takes no medications or dietary supplements. She is 165 cm (5 ft 5 in) tall and weighs 50 kg (110 lb); BMI is 18 kg/m². Vital signs are within normal limits. Examination shows no abnormalities. Supplementation with which of the following is the **most appropriate recommendation for this patient**?

- ☐ A) Calcium
- ☐ B) Folic acid
- ☐ C) Magnesium
- ☐ D) Manganese
- ☐ E) Vitamin A

33. A 38-year-old woman has had an 8-day history of malaise, sore throat, nasal congestion, and rhinitis and a 2-day history of temperatures to 40°C (104°F), shortness of breath, and cough productive of yellow mucus. She is alert and oriented. Her temperature is 39.7°C (103.4°F), pulse is 116/min, respirations are 24/min, and blood pressure is 108/72 mm Hg. Breath sounds are decreased at the left apex posteriorly and anteriorly with increased tactile fremitus over that area. Inspiratory and expiratory crackles and rhonchi are heard. There is a normal S₁ and S₂ and no cyanosis, clubbing, or pedal edema. Her leukocyte count is 16,000/mm³. An x-ray of the chest shows a left upper lobe consolidation. Which of the following is the most likely causal organism?

- ☐ A) *Mycobacterium tuberculosis*
- ☐ B) *Mycoplasma pneumoniae*
- ☐ C) *Neisseria meningitidis*
- ☐ D) *Pneumocystis jiroveci* (formerly *P. carinii*)
- ☐ E) *Pseudomonas aeruginosa*
- ☐ F) Respiratory syncytial virus
- ☐ G) Rhinovirus
- ☐ H) *Streptococcus pneumoniae*

34. A 45-year-old man with gout has the sudden onset of extreme pain, swelling, and exquisite tenderness of the right knee; he cannot bear the pressure of wearing slacks. He has not recently taken any medication and previously had intractable diarrhea while taking colchicine. Which of the following is the most appropriate drug for treating this acute episode?

- ☐ A) Allopurinol
- ☐ B) Codeine
- ☐ C) Indomethacin
- ☐ D) Prednisone
- ☐ E) Probenecid

35. A 72-year-old African American man comes to the office because of a 2-day history of mild, vague abdominal pain. He has hypertension, type 2 diabetes mellitus, and osteoarthritis of the knees. His medications are lisinopril, metformin, and acetaminophen. He does not smoke cigarettes or use illicit drugs. He drinks eight glasses of wine weekly. He is a retired hotel desk clerk. He is 178 cm (5 ft 10 in) tall and weighs 79 kg (175 lb); BMI is 25 kg/m². His temperature is 37.1°C (98.8°F), pulse is 92/min, respirations are 16/min, and blood pressure is 156/97 mm Hg. Abdominal examination shows a pulsatile midline mass 7 cm above the umbilicus. No other abnormalities are noted. Laboratory studies show a hemoglobin A_{1c} of 8.4% and serum total cholesterol concentration of 260 mg/dL. Which of the following factors is most predictive of future complications of this patient's condition?

- ☐ A) Alcohol consumption
- ☐ B) Blood pressure
- ☐ C) Ethnicity
- ☐ D) Hemoglobin A_{1c} measurement
- ☐ E) Serum total cholesterol concentration

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Family Medicine Self AssessmentTime Remaining:
15 min 52 sec

36. A 32-year-old woman comes to the physician because of a 3-month history of frequent loss of urine. During this time, she has had loss of urine at rest and with activity. She wears an absorbent pad each night; it is soaked by morning. She has difficulty initiating her urinary stream, which is weak. She has had six urinary tract infections during the past year. She has had type 1 diabetes mellitus since the age of 6 years. Physical examination shows no abnormalities. A cough stress test is positive. Her postvoid residual volume is 800 mL. Urinalysis shows no abnormalities. Which of the following is the most likely diagnosis?

- ☐ A) Functional incontinence
- ☐ B) Overflow incontinence
- ☐ C) Stress incontinence
- ☐ D) Urge incontinence
- ☐ E) Urinary tract infection
- ☐ F) Vesicovaginal fistula

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Family Medicine Self AssessmentTime Remaining:
14 min 38 sec

37. A 20-year-old woman comes to the office for a routine health maintenance examination. She says she feels well. She has no history of serious illness or sexually transmitted infection. Her only medication is an oral contraceptive, for which she requests a refill. She has had five lifetime sexual partners. During the past 4 months, she has been monogamous with one male partner. She has never used condoms. Vital signs are within normal limits. Examination shows no abnormalities. In addition to refilling the oral contraceptive prescription, which of the following is the most appropriate screening for cervical cancer?

- ☐ A) High-risk human papillomavirus (HPV) genotyping only
- ☐ B) Pap smear and high-risk HPV genotyping
- ☐ C) Pap smear with reflex HPV testing
- ☐ D) Pap smear only
- ☐ E) No screening is indicated at this time



Previous



Next



Lab Values



Calculator



Review



Help



Pause

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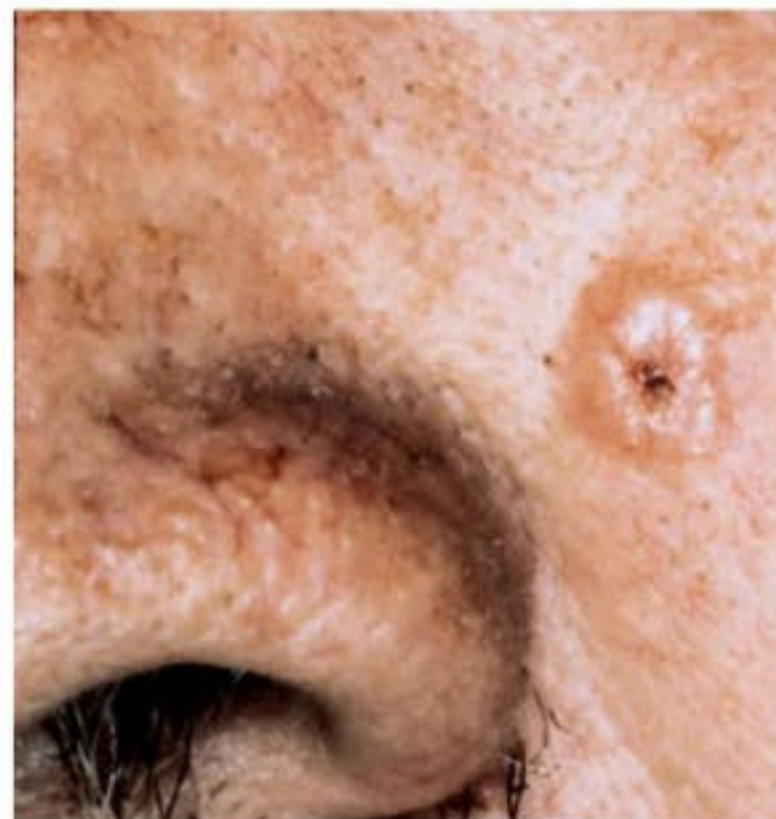
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Family Medicine Self AssessmentTime Remaining:
12 min 53 sec

38. A 28-year-old woman, who was diagnosed with type 2 diabetes mellitus 1 month ago, comes to the office as a new patient. She recently moved to the area. The diagnosis was made based on a 2-week history of excessive urination and a nonfasting serum glucose concentration of 225 mg/dL. She is currently asymptomatic. She has no other history of serious illness. Her only medication is metformin. Vital signs are within normal limits. Examination shows no abnormalities. Today, her hemoglobin A_{1c} is 6.1%, and nonfasting serum glucose concentration is 135 mg/dL. Which of the following is the most appropriate recommendation for this patient to prevent disease-related morbidity?

- ☐ A) Annual eye examination
- ☐ B) Exercise stress test
- ☐ C) Herpes zoster vaccination
- ☐ D) Monthly determination of hemoglobin A_{1c}
- ☐ E) Pneumococcal vaccination

39. A 5-year-old boy is brought to the office by his mother because of a 6-month history of moderate right upper abdominal pain that occurs 15 to 60 minutes after he eats. The episodes occur once or twice weekly and resolve in less than 1 hour. He otherwise is asymptomatic. He has no history of serious illness and receives no medications. His mother says he eats three fast-food meals weekly. Growth and development are appropriate for age. Vital signs are within normal limits. The abdomen is nontender. The spleen tip is palpated 2 cm below the left costal margin. Ultrasonography of the abdomen shows 20 gallstones measuring 3 mm to 10 mm. His hemoglobin concentration is 7.7 g/dL (N=11.5–14.5). A blood smear shows spherocytes. Which of the following is the most likely cause of this patient's condition?

- ☐ A) Change in the shape of red blood cells
- ☐ B) A fat-rich diet
- ☐ C) Presence of antiplatelet IgG antibodies
- ☐ D) Production of autoantibodies directed against the red blood cell membrane
- ☐ E) Switch from glutamic acid to valine in the globin chain



40. A 50-year-old man comes to the physician because of a nonitchy, painless, gradually enlarging lesion on his face that he first noticed 6 months ago. He has no history of serious illness and takes no medications. Vital signs are within normal limits. A photograph of the lesion is shown. Examination shows no other abnormalities. Which of the following is the most likely diagnosis?
- A) Actinic keratosis
 - B) Basal cell carcinoma
 - C) Keratoacanthoma
 - D) Seborrheic keratosis
 - E) Squamous cell carcinoma

41. A 16-year-old girl is brought to the office by her mother because of a 3-week history of nausea without associated vomiting. The patient's mother waits outside the examination room during the evaluation. The patient has no history of serious illness and takes no medications. Since menarche at age 11 years, her menstrual periods have occurred at irregular 30- to 60-day intervals; her last menstrual period was 2 months ago. The patient reports that she has been dating her 18-year-old boyfriend for the past 5 months; they are sexually active and use condoms inconsistently. Temperature is 36.8°C (98.4°F), pulse is 88/min, respirations are 16/min, and blood pressure is 98/62 mm Hg. Physical examination shows no abnormalities. Urine pregnancy test is positive. While discussing the results with the patient, she states that she does not know what to do. Which of the following is the most important question for the physician to ask the patient during the conversation?
- ☐ A) "Do you want me to contact child protective services?"
 - ☐ B) "Have you considered placing the child up for adoption?"
 - ☐ C) "How do you think your boyfriend will react to this news?"
 - ☐ D) "Is this a situation we could discuss with your mother?"
 - ☐ E) "Would you like a confidential referral to discuss abortion services?"

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Family Medicine Self AssessmentTime Remaining:
8 min 13 sec

42. A previously healthy 82-year-old woman comes to the physician because of generalized weakness and a sore tongue for 1 month. She lives by herself. Her diet has consisted mainly of tea and homemade bread since her husband died 6 months ago. Examination shows pallor, mild jaundice, and a smooth, red tongue. There is no lymphadenopathy or organomegaly. Neurologic examination shows no focal findings. Laboratory studies show:

Hematocrit	26%
Mean corpuscular volume	120 μm^3
Leukocyte count	2500/ mm^3
Reticulocyte count	0.2%
Platelet count	20,000/ mm^3

This patient most likely has which of the following nutritional deficiencies?

- ☐ A) Folic acid
- ☐ B) Niacin
- ☐ C) Vitamin B₁ (thiamine)
- ☐ D) Vitamin B₆
- ☐ E) Vitamin B₁₂ (cobalamin)

43. An 82-year-old woman with dementia, Alzheimer type, is brought to the physician by her daughter for recommendations about driving restrictions. Since the patient moved in with her daughter 3 weeks ago, they have had several arguments because her daughter has not allowed the patient to drive. The patient last drove a car 2 years ago. She has no history of motor vehicle collisions. The patient's husband provided her daily care until he died of a myocardial infarction 3 weeks ago. The patient's only medication is galantamine, which she has taken for 4 years. Physical examination shows no abnormalities. Mental status examination shows a sad mood and reactive affect. She reports no suicidal thoughts. She is able to correctly repeat six digits forward, and she recalls one of three objects after 5 minutes; she is unable to correctly copy a rectangle or two intersecting pentagons. Which of the following is the most appropriate recommendation regarding this patient's driving?
- ☐ A) The patient should be allowed to drive only with supervision
 - ☐ B) The patient should be allowed to drive short distances only
 - ☐ C) The patient should be allowed to drive until her driving skills are formally assessed
 - ☐ D) The patient should not have access to car keys until her driving skills are formally assessed
 - ☐ E) The patient's driver license should be revoked

44. A 4-week-old male infant is brought to the office by his parents because of a 1-week history of "increasing spit-up." His parents report that the patient will forcefully spit up within a few minutes after every feeding. He has a history of mild breast-feeding jaundice which resolved spontaneously. The patient was born at term following an uncomplicated spontaneous vaginal delivery to a primigravid mother. Vital signs are within normal limits. Palpation over the right upper abdominal quadrant discloses a 1.5-cm mass. Which of the following is the most appropriate next step to determine the cause of this patient's symptoms?

- ☐ A) Anteroposterior and decubitus x-rays of the abdomen
- ☐ B) Barium enema with air contrast
- ☐ C) Complete blood count and comprehensive metabolic panel
- ☐ D) Ultrasonography of the abdomen
- ☐ E) Observation of the infant while feeding and spitting up

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Family Medicine Self AssessmentTime Remaining:
5 min 28 sec

45. A 24-year-old primigravid woman at 10 weeks' gestation comes to the office for her first prenatal visit. She has no history of serious illness, and her only medication is a prenatal vitamin. Her childhood immunization history is unknown. Vital signs are within normal limits. Examination shows a uterus consistent in size with a 10-week gestation. Which of the following is the most appropriate vaccine to administer at this time?
- ☐ A) Hepatitis B
 - ☐ B) Human papillomavirus
 - ☐ C) Measles-mumps-rubella
 - ☐ D) Tetanus-diphtheria-acellular pertussis
 - ☐ E) Varicella

46. A 9-year-old girl is brought to the physician because of daily expiratory wheezing for 2 weeks. She has also had an occasional cough during exercise and at night. She has had asthma for 2 years treated with inhaled albuterol. Examination shows bilateral expiratory wheezes. Which of the following is the most appropriate next step in pharmacotherapy?

- ☐ A) Inhaled corticosteroids
- ☐ B) Inhaled cromolyn sodium
- ☐ C) Oral leukotriene inhibitor
- ☐ D) Oral prednisone
- ☐ E) No additional pharmacotherapy indicated



47. A 63-year-old man comes to the clinic because of a 4-day history of a severely itchy rash that started on his right hand and forearm but has since spread to his left hand. This morning, he noticed some "spots" around his waistline. Most of the spots are red bumps, but some of the spots now have turned into scabs. The patient's new roommate, who moved into his apartment a few weeks ago, has had a similar itchy rash. The patient has no other history of serious illness and takes no medications. Vital signs are within normal limits. A photograph of the patient's right hand is shown; similar findings are noted over the waistline. There is no crusting or spreading erythema. He has no rash elsewhere. Which of the following is the most appropriate pharmacotherapy at this time?

- A) Oral acyclovir
- B) Oral cephalexin
- C) Oral prednisone
- D) Topical permethrin
- E) Topical triamcinolone

48. A previously healthy 47-year-old woman comes to the physician because of a 1-week history of low back pain that radiates to the left buttock and along the lateral side of the left leg. Her symptoms began 1 day after moving furniture in her home. She has not had fever, chills, constipation, or urinary incontinence. Examination shows diffuse tenderness over the paraspinal muscles of the lumbosacral area and no tenderness over the spine. Range of motion of the left lower extremity is limited by pain. Deep tendon reflexes are normal, muscle strength is intact, and sensory examination shows no abnormalities. Which of the following is the most appropriate next step in management?

- ☐ A) Bed rest for 1 week
- ☐ B) CT scan of the lumbosacral spine
- ☐ C) Epidural corticosteroid injection
- ☐ D) Oral ibuprofen therapy
- ☐ E) Oral prednisone therapy



49. A 55-year-old man comes to the office because of a 2-week **history of a recurrent and severely itchy diffuse rash**. The patient reports that the rash typically resolves within 3 to 4 hours and is most severe over his back. He has not had fever, shortness of breath, abdominal pain, or any other symptoms. Use of diphenhydramine during the episodes provides moderate relief. He has no history of serious illness and takes no routine medications. Vital signs are within normal limits. The patient does not currently have a rash but shows a previously taken photograph of his back. Which of the following is the most appropriate diagnostic study at this time?

- A) C1-esterase inhibitor testing
- B) Complete blood count with leukocyte differential
- C) Radioallergosorbent testing
- D) Serum C3 and C4 concentrations
- E) No further diagnostic studies are indicated

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Family Medicine Self Assessment

Time Remaining:

1 min 15 sec

50. A 37-year-old woman, gravida 3, para 3, comes to the physician for a routine health maintenance examination. One year ago, her serum cholesterol concentration was 260 mg/dL, and she was started on a low-fat diet. She has no history of serious illness. Her Pap smears during the past 10 years have shown no abnormalities. Her last Pap smear was 1 year ago. Her mother has type 2 diabetes mellitus, and her father died of colon cancer at the age of 62 years. The patient does not smoke cigarettes or drink alcohol. She has been married for 15 years. She is 157 cm (5 ft 2 in) tall and weighs 77 kg (170 lb); BMI is 31 kg/m². Her pulse is 64/min, and blood pressure is 120/60 mm Hg. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in management?

- ☐ A) Colonoscopy
- ☐ B) 5-Hour glucose tolerance test
- ☐ C) Mammography
- ☐ D) Measurement of serum cholesterol concentration
- ☐ E) Pap smear